DOSING & ADMINISTRATION

Dosing and administration

Starting doses, dose range and dose adjustment for ZYPREXA coated tablets in adults.[1]

Formulations | Starting doses | Stopping treatment

Formulations

This site covers ZYPREXA coated tablets. The injectable formulations have separate prescribing information.



THIS PAGE

ZYPREXA coated tablets (oral olanzapine)

All dosing and safety content below refers to the oral formulation.



SEPARATE PRESCRIBING INFORMATION

ZYPREXA powder for solution for injection

Short-term intramuscular use for agitation. Different dosing and precautions.[2]



SEPARATE PRESCRIBING INFORMATION

ZYPADHERA (olanzapine pamoate)

Prolonged-release injection with its own dosing and post-injection observation requirements.[3]



SEPARATE ORAL FORMULATION

ZYPREXA Velotab orodispersible tablets

Orodispersible tablets are a separate oral presentation. Dosing and safety on this page refer to ZYPREXA coated tablets. Confirm the Velotab product information before prescribing that formulation.

Starting doses and dose range

Recommended starting doses for oral olanzapine in adults.[1]

Table 1. Recommended starting dose by treatment setting

Approved treatment setting	Starting dose
Schizophrenia	10 mg once daily
Moderate to severe manic episode, monotherapy	15 mg single daily dose
Moderate to severe manic episode, combination therapy	10 mg once daily
Prevention of recurrence in bipolar disorder following response to olanzapine for mania	10 mg once daily

The daily dose range is **5–20 mg**, individualised for each patient. Dose adjustments should be based on clinical assessment. An increase above the recommended starting dose should occur only after appropriate clinical reassessment and generally at intervals of not less than 24 hours.[1]

For patients who have been receiving olanzapine for a manic episode, therapy for preventing recurrence continues at the same dose. If a new manic, mixed or depressive episode occurs, olanzapine treatment should be continued (with dose optimisation as needed), with supplementary therapy to treat mood symptoms, as clinically indicated.[1]

Olanzapine can be given without regard to meals, as absorption is not affected by food.

Special populations

Older patients (65 years and over)

A lower starting dose (5 mg/day) is not routinely indicated but should be considered when clinical factors warrant.[1]

Renal and/or hepatic impairment

A lower starting dose (5 mg) should be considered. In moderate hepatic insufficiency (cirrhosis, Child-Pugh Class A or B), the starting dose should be 5 mg and only increased with caution.[1]

Factors that may slow metabolism

When more than one factor is present (female gender, geriatric age, non-smoking status), consider decreasing the starting dose. Dose escalation should be conservative.[1]

Smokers

Starting dose and range need not be routinely altered. Metabolism may be induced by smoking; clinical monitoring is recommended and a dose increase may be considered if necessary.[1]

Children and adolescents

Not recommended below 18 years of age due to a lack of data on safety and efficacy.[1]

Stopping treatment

Acute symptoms such as sweating, insomnia, tremor, anxiety, nausea or vomiting have been reported rarely when olanzapine is stopped abruptly.[1]

Gradual tapering of the dose should be considered when discontinuing olanzapine. Treatment changes should be managed by the treating healthcare professional.[1]

Before prescribing

Check contraindications, warnings and the monitoring schedule.

Contraindications → **Monitoring schedule →** **Interactions →**

← Previous screen: **Mechanism of action**

Next screen: **Safety & monitoring** →

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